



YMNOTES MEDICAL · COMPANION HANDOUT

YMnotes Digestive System

After Mid-Module Extras

Points found in **GIT Others** that are missing from your main book, placed where they fit in your chapters. Use this after you finish each chapter.

3

additions · chapters 15–40

17

earlier-chapter additions

6

points to check



HOW TO USE

About this handout

This handout goes with **YMnotes Digestive System • Version 3**. It contains only what **GIT Others** adds to your book: a missing fact, distinction, mechanism or clinical detail within a topic your book already covers. Anything already in your book, even in different words, has been left out.

Reading a reference

Book p. 196 is the **printed page number** at the foot of your book's page.

1st-half PDF p. 199 is the page number your PDF viewer shows in 4_5875397832227168769.PDF. **2nd-half PDF** means 4_5875397832227168769(1).PDF.

GIT Others p. 20 is the page in that source. Its PDF page and printed page are the same.

Reading an entry

Fits after names the heading in your book where the point belongs.

Each entry has a label in the margin: **KEY FACT**

MECHANISM **DISTINCTION** **CLINICAL**

CHECK entries with a dashed border mark places where the sources disagree. Both versions are quoted and neither is chosen for you.

Where the mid-module split falls

File	Printed pages	How to convert	Chapters
1st half · 4_5875397832227168769.PDF (200 PDF pages)	1–197	printed = PDF page – 3	1–14, and the start of 15
2nd half · 4_5875397832227168769(1).PDF (238 PDF pages)	197–433	printed = PDF page + 196 (PDF p. 238 is the closing page, with no number)	the rest of 15, then 16–40

The split falls **inside Chapter 15, Colorectal Cancer**, which starts on p. 195. Printed page 197 appears in both files. The page numbers your PDF viewer shows are not the book's printed page numbers, so always use the printed number in this handout.

Part A covers Chapters 15–40, the material after the mid-module. **Part B** holds the additions for earlier chapters, because GIT Others covers mainly first-half topics (drug treatment of peptic ulcer, GIT tumour pathology, foodborne infection and parasitology).





CONTENTS

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Tap an entry to go to it. The page numbers on the right are this handout's own pages. The book page ranges are printed pages in your main book.

PART A · AFTER MID-MODULE CHAPTERS

15	Colorectal Cancer Surgery · Pathology	Book pp. 195–209 · 4 entries	4
25	Pyogenic Liver Abscess Surgery · Internal Medicine · Parasitology	Book pp. 324–331 · 1 entry	5
—	Chapters 16–24, 26–40: checked, nothing to add Every after-mid chapter compared against GIT Others	Book pp. 210–433	5

PART B · EARLIER-CHAPTER ADDITIONS FOUND IN GIT OTHERS

03	The Stomach and Duodenum Internal Medicine · Pharmacology · Surgery	Book pp. 33–69 · 4 entries	6
04	Pathology of GIT Tumours Pathology · Surgery	Book pp. 70–75 · 8 entries	7
07	Foodborn Infection Microbiology · Internal Medicine	Book pp. 106–114 · 8 entries	8
14	Polyposis of Colon Surgery · Pathology	Book pp. 189–194 · 1 entry	9

REVIEW

!	Points to check: sources disagree All CHECK entries in one list	10
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PART A

After Mid-Module Chapters

Additions for the chapters in the second half of your book, in book order. Chapter 15 starts in the first-half file (p. 195) and continues into the second-half file, so it is included here in full.

15

Colorectal Cancer

Surgery · Pathology · Main book pp. 195–209

Split chapter: pp. 195–197 are in the first-half file (PDF pp. 198–200); pp. 197–209 are in the second-half file (PDF pp. 1–13). Page 197 appears in both. Teach after finishing this chapter.

Fits after: 3 - Microscopic picture → Adenocarcinoma · **Book p. 196** (1st half PDF p. 199)

Adenocarcinoma: 98% of large-bowel cancers

Your book says “more than 90%” of colorectal carcinomas are adenocarcinomas. GIT Others gives a precise figure: **98%** of all cancers of the large intestine are adenocarcinomas.

KEY FACT

GIT Others p. 19
A15-1
ledger TU-30

Fits after: 3 - Microscopic picture → Mucinous adenocarcinoma (variant) · **Book p. 196** (1st half PDF p. 199)

Two kinds of mucoid (mucinous) carcinoma

- ◆ **Mucinous adenocarcinoma:** abundant *extracellular* mucin (your book: >50% mucinous material).
- ◆ **Signet ring cell carcinoma:** the mucin stays *inside* the malignant cell and pushes the nucleus to one side (eccentric nucleus).

DISTINCTION

GIT Others p. 20
A15-2
ledger TU-36

Fits after: Aetiology → 1 - Predisposing factors · **Book p. 196** (1st half PDF p. 199)

Add irradiation to the predisposing factors

GIT Others lists **irradiation** as a predisposing factor for colorectal adenocarcinoma, along with adenoma, familial polyposis, IBD and gene mutations. It is not in your book’s list.

RISK FACTOR

GIT Others p. 19
A15-3
ledger TU-32

Fits after: Pathology of colorectal carcinoma → Age · **Book p. 196** (1st half PDF p. 199) · also Ch 4, p. 74 (1st-half PDF p. 77)

Age at presentation: 50–70 or 60–79 years?

- ◆ GIT Others p. 19: “Age of **50–70** years.”
 - ◆ GIT Others p. 20 and your book Ch 4 p. 74: “between **60 and 79** years of age”.
 - ◆ Your book Ch 15 p. 196: “old age, however no age is immune”.
- GIT Others contradicts itself. Ask your teacher which range to learn.

CHECK

Sources differ
GIT Others p. 19, 20
A15-D1
ledger TU-31



25

Pyogenic Liver Abscess

Surgery · Internal Medicine · Parasitology · Main book pp. 324–331

Second-half file, PDF pp. 128–135. The amoebic liver abscess section is on pp. 329–331. Teach after finishing this chapter.

Fits after: Amebic liver abscess → Investigations → Lab (CBC) · **Book p. 331** (2nd half PDF p. 135) · also Ch 9, p. 138 (1st-half PDF p. 141)

Blood count in amoebic liver abscess

- ◆ Your book Ch 25 p. 331: “CBC: Leukocytosis and **eosinophilia**”.
- ◆ GIT Others p. 47, and your book Ch 9 p. 138: CBC shows **polymorphonuclear leucocytosis**, which helps separate amoebic liver abscess from viral hepatitis. Neither mentions eosinophilia for amoebic liver abscess.

Both agree on leucocytosis. Check with your teacher whether eosinophilia should be learned.

CHECK

Sources differ

GIT Others p. 47

A25-D1

ledger PA-29

Chapters 16–40 with nothing to add

Each of these chapters was compared with GIT Others. None has a missing point that belongs in it.

GIT Others touches the topic, but your book already covers it:

Ch	Chapter	Book pages	Where your book already has it
19	Acute Hepatitis	pp. 251–263	Leptospirosis and yellow fever liver involvement is already in Ch 7 (pp. 113–114).
22	Liver Cirrhosis	pp. 280–307	Hepatosplenic schistosomiasis is already on p. 283 and in Ch 9.
23	Portal Hypertension	pp. 308–317	Schistosomal periportal fibrosis (p. 310) and PPIs for bleeding varices (Ch 3 p. 47) are already covered.
24	Hydatid Cyst	pp. 318–323	GIT Others only names hydatid disease as a differential of amoebic liver abscess, which Ch 9 already does.
35	Obstructive Jaundice	pp. 389–398	Biliary Ascaris is already on p. 390; the Ascaris details in GIT Others are all in Ch 9.

No matching material in GIT Others:

16	Anorectal Diseases	pp. 210–230	31	Acute Hepatic Failure	pp. 361–366
17	Hepatology and Hepatic Surgery	pp. 231–235	32	Biliary System Anatomy	pp. 367–372
18	Liver Disease Assessment	pp. 236–250	33	Gallbladder Diseases	pp. 373–375
20	Chronic Hepatitis B	pp. 264–273	34	Gallstones and Cholecystitis	pp. 376–388
21	Autoimmune Hepatitis	pp. 274–279	36	Acute Pancreatitis	pp. 399–406
26	Hepatocellular Carcinoma	pp. 332–336	37	Cancer Pancreas	pp. 407–412
27	Ascites	pp. 337–344	38	Acute Abdomen	pp. 413–417
28	Renal Impairment in Cirrhosis	pp. 345–348	39	Abdominal Trauma	pp. 418–424
29	Jaundice	pp. 349–354	40	Splenic Trauma	pp. 425–433
30	Hepatic Encephalopathy	pp. 355–360			





PART B · NOT PART OF THE AFTER-MID-MODULE CHAPTERS

Earlier-Chapter Additions Found in GIT Others

Most of GIT Others covers chapters from the first half of your book (before the mid-module). These points are missing from those chapters. They are kept separate from Part A and follow your book's chapter numbers. Use them when revising the earlier chapters.

03

The Stomach and Duodenum

Internal Medicine · Pharmacology · Surgery · Main book pp. 33–69

First-half file, PDF pp. 36–72. GIT Others pp. 1–6 (drug treatment of gastritis and peptic ulcer) repeat your book's pp. 44–50 almost word for word. Teach after finishing this chapter.

Fits after: Pharmacology → 1. Proton Pump Inhibitors → Adverse effects · **Book p. 45** (1st half PDF p. 48)

Long-term PPI use and bone

Osteoporosis is a possible complication of chronic (long-term) PPI use. Add it to the PPI adverse effects, after vitamin B12 deficiency.

ADVERSE EFFECT

GIT Others p. 11 (MCQ 24; answer key p. 12)

B3-1

ledger PH-26

Fits after: NSAID-induced ulcer → prophylaxis (misoprostol) · **Book p. 37** (1st half PDF p. 40) · also p. 48 (1st-half PDF p. 51)

Misoprostol: PGE1 or PGE2 analogue?

- ◆ Your book p. 37: “a synthetic prostaglandin **E₂** analogue”.
- ◆ Your book p. 48 and GIT Others p. 4: “Synthetic **PGE1** analog”.

Your two pages disagree. GIT Others agrees with p. 48.

CHECK

Book inconsistent

GIT Others p. 4

B3-D1

ledger PH-15

Fits after: H. pylori eradication → rescue (quinolone) therapy · **Book p. 41** (1st half PDF p. 44) · also p. 48 (1st-half PDF p. 51)

Levofloxacin dose in the rescue regimen

- ◆ Your book p. 41: “Levofloxacin **500 mg once daily** + PPI + Amoxicillin”.
- ◆ Your book p. 48 and GIT Others p. 5: “levofloxacin **250 or 500 mg twice a day** for 10 days” (with PPI + amoxicillin 1 g twice a day).

Your two pages disagree. GIT Others agrees with p. 48.

CHECK

Book inconsistent

GIT Others p. 5

B3-D2

ledger PH-16

Fits after: Cancer stomach → Site · **Book p. 62** (1st half PDF p. 65)

Commonest site of gastric cancer: how common?

- ◆ Your book p. 62: antrum/pyloric area **60%**, body 15%, fundus 25%.
- ◆ GIT Others p. 15: pyloric region **50%**, then **lesser curvature 25%**, but it can occur anywhere in the stomach.

Both sources agree the pyloric (antral) region is the commonest site. They give different percentages and a different second site.

CHECK

Sources differ

GIT Others p. 15

B3-D3

ledger TU-13



04

Pathology of GIT Tumours

Pathology · Surgery · Main book pp. 70–75

First-half file, PDF pp. 73–78. Most of GIT Others pp. 13–21 already appears on these pages. Teach after finishing this chapter.

Fits after: Carcinoma of esophagus → Histopathology → 1. Squamous cell carcinoma · **Book p. 71** (1st half PDF p. 74)

What esophageal SCC looks like under the microscope

It is made of **islands of neoplastic squamous cells**. Differentiated tumours also show **epithelial cell nests**.

MICROSCOPY

GIT Others p. 14
B4-1
ledger TU-06

Fits after: Carcinoma of esophagus → effects and complications · **Book p. 71** (1st half PDF p. 74)

A malignant ulcer can bleed fatally

As well as dysphagia and haematemesis, **perforation of the malignant ulcer can cause fatal haemorrhage**.

COMPLICATION

GIT Others p. 14
B4-2
ledger TU-07

Fits after: Gastric adenocarcinoma → Predisposing factors / intestinal type “on top of intestinal metaplasia” · **Book p. 72** (1st half PDF p. 75)

The H. pylori pathway to gastric cancer

H. pylori infection → gastritis → atrophic gastritis → **intestinal metaplasia** → **dysplasia** → cancer.

GIT Others calls intestinal metaplasia a **precancerous (dysplastic) lesion** of the stomach.

MECHANISM

GIT Others p. 15
B4-3
ledger TU-12

Fits after: Gastric adenocarcinoma → “It may take one of two types” · **Book p. 72** (1st half PDF p. 75)

Name and frequency of the two types

The intestinal and diffuse types make up the **Lauren classification**: intestinal type **53%**, diffuse type **33%**, unclassified **14%**.

CLASSIFICATION

GIT Others p. 16
B4-4
ledger TU-16

Fits after: Late gastric cancer → (2) Malignant ulcer · **Book p. 72** (1st half PDF p. 75)

The malignant gastric ulcer

The ulcer is typically **more than 5 cm** across, with an everted edge, a **necrotic floor** and a **firm base**.

GROSS PATHOLOGY

GIT Others p. 16
B4-5
ledger TU-15

Fits after: Spread of gastric carcinoma → Blood spread · **Book p. 73** (1st half PDF p. 76)

Blood spread goes beyond the liver

Blood-borne metastases go to the liver and also to the **lung, bone and brain**.

SPREAD

GIT Others p. 16
B4-6
ledger TU-19

Fits after: Neoplastic adenomatous polyp of the colon · **Book p. 73** (1st half PDF p. 76)

Colonic adenomas run in families

There is a **well-defined familial predisposition** to colonic adenomas. They predispose to colorectal carcinoma.

RISK FACTOR

GIT Others p. 17
B4-7
ledger TU-23





Fits after: Tubular adenoma / Villous adenoma · **Book p. 73** (1st half PDF p. 76)

Tubular vs villous adenoma: size and cancer risk

	Tubular adenoma	Villous adenoma
Size	Small, <1 cm	Large, >2 cm
Number	Multiple	Solitary
Dysplasia / cancer	Rare	High (about 40%)

GIT Others' table gives the 40% for adenomas >4 cm; its text gives it for villous adenomas in general.

DISTINCTION

GIT Others p. 18
B4-8
ledger TU-25

07

Foodborn Infection

Microbiology · Internal Medicine · Main book pp. 106–114

First-half file, PDF pp. 109–117. GIT Others pp. 22–33 repeat these pages almost word for word. Only small laboratory and control details are new. Teach after finishing this chapter.

Fits after: Salmonella → acute gastroenteritis (contaminated eggs) · **Book p. 107** (1st half PDF p. 110)

Where Salmonella gastroenteritis comes from

Poultry are the primary reservoir of Salmonella causing food-borne gastroenteritis. Contaminated eggs are the main food source.

RESERVOIR

GIT Others p. 22
B7-1
ledger FI-02

Fits after: Salmonella → Culture and Biochemical reactions · **Book p. 107** (1st half PDF p. 110)

Salmonella culture and sugar fermentation

- ◆ A **facultative anaerobe** that grows well at **37 °C in 24 hours**. Colonies on MacConkey are **pale (yellow)** because it does not ferment lactose.
- ◆ Most species ferment sugars with **acid and gas**. **S. typhi makes acid only** (no gas).

LAB

GIT Others p. 22
B7-2
ledger FI-03, FI-05

Fits after: Enteric fever → Laboratory diagnosis → A. Isolation (first week) · **Book p. 109** (1st half PDF p. 112)

How the first-week blood culture is done

Blood is cultured in **fluid (broth) media**, then **subcultured on solid media**, mainly MacConkey, at 37 °C.

LAB

GIT Others p. 24
B7-3
ledger FI-10

Fits after: Enteric fever → Prevention and Control (before "1- Immunization") · **Book p. 109** (1st half PDF p. 112)

Control starts with food handlers

Food handlers are examined to detect carriers. Carriers are treated or kept away from handling food. GIT Others lists this before immunization.

PUBLIC HEALTH

GIT Others p. 25
B7-4
ledger FI-16

Fits after: Salmonella food poisoning → A. S. typhimurium · **Book p. 110** (1st half PDF p. 113)

S. typhimurium food poisoning is self-limiting

After lasting 2–5 days, **it usually resolves on its own (spontaneous cure)**.

COURSE

GIT Others p. 25
B7-5
ledger FI-18





Fits after: Campylobacter → Laboratory diagnosis · **Book p. 111** (1st half PDF p. 114)

Campylobacter laboratory details

- ◆ A **rectal swab** is an acceptable specimen, collected with care to avoid oxygen exposure.
- ◆ Darting motility is seen in a wet mount under **dark-field illumination**.
- ◆ Isolated colonies are identified by **biotyping, serotyping and phage typing**.

LAB

GIT Others p. 27

B7-6

ledger FI-23

Fits after: Brucella → Culture · **Book p. 111** (1st half PDF p. 114)

Brucella needs CO₂

Brucella is aerobic, but culture needs **10% CO₂** as well as serum-enriched media.

LAB

GIT Others p. 27

B7-7

ledger FI-26

Fits after: Enteric fever → Interpretation of Widal test · **Book p. 109** (1st half PDF p. 112)

Widal test: what a high H titre means

- ◆ Your book p. 109: a high titre to **one** H antigen (≥1:160) suggests recent infection with that type; high O titre **plus more than one** H antigen indicates recent immunization.
- ◆ GIT Others p. 24: “**More than one** H antigen (≥1:160) suggest recent infection with this type of Salmonella **and** recent immunization.”

The GIT Others list here is garbled (its items 1–2 are cut off), so it is not a reliable correction. Ask your teacher before changing what you learned.

CHECK

Sources differ

GIT Others p. 24

B7-D1

ledger FI-13

14

Polyposis of Colon

Surgery · Pathology · Main book pp. 189–194

First-half file, PDF pp. 192–197. Teach after finishing this chapter.

Fits after: Familial polyposis coli → “These are adenomatous polyps” · **Book p. 190** (1st half PDF p. 193)

Polyp type in FAP

In FAP, **most of the polyps are tubular adenomas**.

PATHOLOGY

GIT Others p. 19

B14-1

ledger TU-29

GIT Others also covers Chapter 9 (Parasitic GI Diseases) on pp. 35–47. Every point there is already in your book (pp. 123–138), with the same doses and time periods, so Chapter 9 has no additions.





REVIEW

Points to check

In these places your book and GIT Others disagree, or your book disagrees with itself. Both versions are quoted in the entry and neither is chosen for you. Tap an ID to go to the entry.

Entry	Where	Topic	Main book	Source
A15-D1	Part A · Ch 15	Age at presentation: 50–70 or 60–79 years?	Book p. 196 (1st half PDF p. 199) also Ch 4, p. 74 (1st-half PDF p. 77)	GIT Others p. 19, 20
A25-D1	Part A · Ch 25	Blood count in amoebic liver abscess	Book p. 331 (2nd half PDF p. 135) also Ch 9, p. 138 (1st-half PDF p. 141)	GIT Others p. 47
B3-D1	Part B · Ch 3	Misoprostol: PGE1 or PGE2 analogue?	Book p. 37 (1st half PDF p. 40) also p. 48 (1st-half PDF p. 51)	GIT Others p. 4
B3-D2	Part B · Ch 3	Levofloxacin dose in the rescue regimen	Book p. 41 (1st half PDF p. 44) also p. 48 (1st-half PDF p. 51)	GIT Others p. 5
B3-D3	Part B · Ch 3	Commonest site of gastric cancer: how common?	Book p. 62 (1st half PDF p. 65)	GIT Others p. 15
B7-D1	Part B · Ch 7	Widal test: what a high H titre means	Book p. 109 (1st half PDF p. 112)	GIT Others p. 24

Compiled from GIT Others (47 pages) against both files of YMnotes Digestive System, Version 3 (printed pp. 1–433). Every entry is in the comparison ledger that comes with this handout.

رَبِّ زِدْنِي عِلْمًا

[سورة طه: 114]



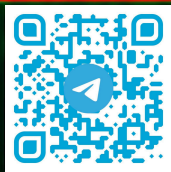
محاضرات فيديو مراجعة شاملة

COMPLETE MEDICAL VIDEO LECTURES

 ENT	 Forensic Medicine	 Special Sense Eye
 Ophthalmology	 Rheumatology	 Orthopedic
 Clinical Toxicology	 Cardiovascular System	 Cardiology
 Respiratory System	 Chest	 Gastrointestinal Tract
 General Surgery	 Special Surgery	 Urinary System
 Pediatric 1	 Pediatrics 2	 Surgery
 Neurology	 Endocrinology	 Obstetrics
 Nephrology	 GIT	 Vascular



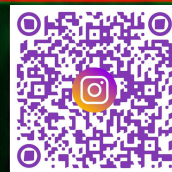
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شرح مبسط
ومنظم



محتوى شامل
ومحدث



مناسب لجميع
مراحل الدراسة



راجع في أي وقت
ومن أي مكان